

COMBINED MEDICAL RECORDS PACKAGE

Motor Vehicle Collision — 10/18/2025
ACTD Owl-Service Bus Incident, South Main Street at Third Avenue

Prepared in response to authorized HIPAA release; contains discharge summaries, follow-up clinic notes, physical therapy discharge, and itemized billing summary for the two passenger patients treated at this facility following the referenced collision.

Package contents:

- Section A — Devin Cho (MRN 004182773)
- Section B — Alicia Mowbray (MRN 004182774)

Records custodian	Renata Wills, RHIA — Director, Health Information Management
Facility	Rivergate Regional Trauma Center, 2100 Medical Center Drive, Rivergate, CA 91766
Package assembled	03/12/2026
Certification	Pages certified as true and correct copies of the medical record maintained in the ordinary course of business.

SECTION A — DEVIN CHO

Cho, Devin

MRN: 004182773 DOB: 09/14/2002 Sex: M/F
Adm: 10/18/2025 02:47 D/C: 10/22/2025 Coverage: On file

Patient Demographics

Patient name	Devin Cho
Date of birth	09/14/2002
Address	245 Elm Avenue, Apt 12, Rivergate, CA 91766
Telephone	(626) 555-0142
Medical record number	004182773
Encounter type	Emergency admission → inpatient
Admission date/time	10/18/2025, 02:47 a.m.
Discharge date	10/22/2025
Length of stay	4 days
Attending (admission)	Priya Ramaswamy, MD — Trauma Surgery
Attending (operative)	Bennett R. Halloran, MD — Orthopedic Trauma
Discharging physician	Priya Ramaswamy, MD — Trauma Surgery

Presenting History (10/18/2025 — Emergency Department)

Patient is a 23-year-old nursing student who arrived by ground ambulance from the scene of a bus collision at South Main Street and Third Avenue at approximately 02:35 a.m. on 10/18/2025. Per EMS handoff, patient was a seated passenger on an ACTD Owl-service bus (Route 512) when the bus left its lane and struck a fixed roadside object. Patient reports being thrown forward and to the right at the moment of impact, striking the right lower leg against a metal seat stanchion and the head against the interior wall of the bus. Brief loss of consciousness estimated by co-passenger at less than 60 seconds; patient recalls the moments before impact and the arrival of emergency personnel but has no clear memory of the interval in between.

Chief complaints at presentation: severe right lower-extremity pain with visible deformity; headache; nausea; brief loss of consciousness with residual confusion.

Denies chest pain, shortness of breath, abdominal pain, or numbness/tingling in the extremities other than at the injured leg. No prior fractures. No prior head injury. No anticoagulant use. No known drug allergies. Immunizations current.

Emergency Department Assessment

Arrival vitals: BP 138/86, HR 104, RR 20, SpO₂ 98% on room air, Temp 36.8 °C. Glasgow Coma Scale 14 (E4 V4 M6) on arrival, improving to 15 within 30 minutes. Alert and oriented to person and place; disoriented to time on initial exam; fully oriented on reassessment at 03:20.

Focused examination:

- **Head:** 4 cm parietal scalp abrasion, right side; no palpable step-off; no hemotympanum; pupils equal and reactive. No cervical spine tenderness on palpation; range of motion deferred pending imaging.
- **Chest / abdomen:** nontender; no seatbelt sign (no seatbelt in service on transit bus); breath sounds equal.
- **Right lower extremity:** obvious mid-shaft deformity; skin intact (no bony fragment through the skin); moderate swelling; distal pulses 2+ dorsalis pedis and posterior tibial; capillary refill under 3 seconds; sensation intact to light touch throughout the foot; motor examination limited by pain.

- **Left lower extremity:** unremarkable.
- **Upper extremities:** contusion to right forearm; no deformity; full range of motion.

Imaging (10/18/2025)

- **CT head without contrast:** no intracranial hemorrhage; no midline shift; no skull fracture. Impression: negative for acute intracranial pathology; findings consistent with concussion in the setting of witnessed brief loss of consciousness.
- **CT cervical spine:** no acute fracture or malalignment.
- **Radiograph, right tibia/fibula (AP and lateral):** closed, displaced, mid-shaft fracture of the right tibia with associated fracture of the right fibula. Approximately 8 mm shortening and 12° apex-anterior angulation. No intra-articular extension.
- **Chest radiograph:** clear.

Emergency Department Diagnoses

1. Closed displaced fracture, right tibia, mid-shaft (S82.202A)
2. Closed fracture, right fibula (S82.402A)
3. Concussion, Grade 2, with brief loss of consciousness (S06.0X1A)
4. Scalp abrasion, right parietal region
5. Contusion, right forearm

Hospital Course

Patient was admitted to the trauma inpatient service under Dr. Ramaswamy following orthopedic consultation with Dr. Halloran. The right lower extremity was placed in a long-leg posterior splint with the leg elevated; ice was applied; analgesia was managed with acetaminophen, ketorolac, and short-course oral oxycodone titrated to comfort. Neurovascular checks every two hours through the first 24 hours were reassuring throughout.

Neurological status was monitored on the post-concussion protocol. Headache peaked on hospital day 1 and improved with rest, hydration, and acetaminophen. Photophobia resolved by hospital day 2. Serial cognitive screening (Standardized Assessment of Concussion) showed steady improvement from 22/30 on admission to 28/30 by hospital day 3. No focal neurologic deficits developed. Repeat neurologic examination on the morning of hospital day 4 was normal.

Operative intervention (10/19/2025). Patient was taken to the operating room on hospital day 2 for open reduction and internal fixation of the right tibia and fibula. The operative note (B. Halloran, MD, dictated 10/19/2025) documents:

- Procedure: Open reduction and internal fixation (ORIF), right tibia, with intramedullary nail; ORIF, right fibula, with plate and screws.
- Anesthesia: general with peripheral nerve block.
- Estimated blood loss: 150 mL.
- Complications: none.
- Findings: transverse mid-shaft tibia fracture with butterfly fragment; fibula fracture reduced anatomically; alignment restored; wound irrigated and closed in layers.
- Weight-bearing status postoperatively: non-weight-bearing right lower extremity for six weeks; touch-down weight-bearing thereafter as tolerated by follow-up.

Postoperative recovery was uneventful. Pain control transitioned to oral analgesics by postoperative day 1. Deep-vein-thrombosis prophylaxis with enoxaparin was continued through hospitalization. Physical therapy initiated non-weight-bearing transfer training and crutch instruction on postoperative day 2. Occupational therapy provided activities-of-daily-living training given the non-weight-bearing restriction. Patient tolerated advancement of diet without difficulty and voided normally throughout the admission.

Discharge Summary — 10/22/2025

Discharge diagnoses:

1. Status post ORIF, right tibia and fibula, closed displaced mid-shaft fracture (10/19/2025)
2. Concussion, Grade 2, resolving
3. Scalp abrasion, right parietal region, healing
4. Right forearm contusion, resolving

Discharge condition: Stable, ambulating with crutches, non-weight-bearing right lower extremity. Pain controlled on oral analgesics. Neurologically at baseline.

Disposition: Home, with family support.

Medications on discharge:

- Oxycodone 5 mg PO q6h PRN pain, #24 tablets, no refill
- Acetaminophen 500 mg PO q6h PRN pain
- Ibuprofen 600 mg PO q8h with food PRN pain
- Enoxaparin 40 mg SC daily × 14 days (DVT prophylaxis)
- Docusate 100 mg PO BID PRN

Activity and restrictions:

- Non-weight-bearing right lower extremity for six weeks from date of surgery.
- No driving until cleared by orthopedic surgeon.
- Work restriction: eight (8) weeks off from all work and clinical rotations pending re-evaluation, given combined orthopedic recovery and post-concussion recovery.
- Cognitive rest first 10 days: limit screen exposure, no strenuous mental exertion; return to academic coursework gradually as tolerated.
- Physical therapy referral placed, to begin after two-week postoperative visit.

Follow-up appointments arranged:

- Orthopedic clinic (Dr. Halloran): 11/02/2025
- Post-concussion clinic (neurology, Dr. E. Portner): 10/29/2025
- Trauma clinic (Dr. Ramaswamy): 11/06/2025

Return precautions provided for worsening headache, vomiting, focal weakness, calf swelling, fever, wound drainage, or uncontrolled pain.

Electronically signed: Priya Ramaswamy, MD — Trauma Surgery, 10/22/2025, 14:12

Co-signed: Bennett R. Halloran, MD — Orthopedic Trauma, 10/22/2025, 15:04

Orthopedic Follow-Up Notes — Devin Cho

Clinic visit — 11/02/2025 (Dr. Halloran). Two-week postoperative visit. Wound clean, dry, intact; sutures removed. Radiographs show hardware in appropriate position, alignment maintained, early callus formation. Patient reports controlled pain, weaned off oxycodone. Continue non-weight-bearing. Physical therapy authorized to begin week of 11/10/2025 (non-weight-bearing ROM and upper-body strengthening). Return in four weeks.

Clinic visit — 12/01/2025 (Dr. Halloran). Six-week postoperative visit. Radiographs show progressive union of tibia; fibula healed. Patient advanced to touch-down weight-bearing with crutches. Physical therapy plan updated to include partial-weight-bearing gait training. Sleep and cognitive symptoms reported as resolved. Return in four weeks.

Clinic visit — 12/29/2025 (Dr. Halloran). Ten-week postoperative visit. Radiographs show continued bridging callus. Advanced to weight-bearing as tolerated with one crutch, transition to cane over next two weeks. Physical therapy progressing to strengthening and gait retraining. Cleared to return to non-clinical academic coursework. Work restriction extended pending clearance for clinical rotations requiring weight-bearing and lifting.

Clinic visit — 02/09/2026 (Dr. Halloran). Sixteen-week postoperative visit. Union radiographically confirmed. Full weight-bearing without assistive device achieved. Range of motion of knee and ankle within 5° of contralateral side. Strength 4+/5 quadriceps and gastrocnemius, right. Patient cleared to resume clinical nursing rotations with no lifting restriction beyond program standard. Physical therapy to continue through 03/10/2026 for terminal strengthening.

Post-Concussion Clinic Notes — Devin Cho

Clinic visit — 10/29/2025 (Dr. E. Portner, Neurology). Post-concussion evaluation. Symptoms limited to mild intermittent headache and fatigue. SCAT-5 within age-adjusted norms. Cleared for graduated return to cognitive activity. Return in three weeks.

Clinic visit — 11/19/2025 (Dr. E. Portner, Neurology). Symptom-free × 10 days. Neurological examination normal. Discharged from post-concussion follow-up with return precautions.

Physical Therapy Discharge Summary — Devin Cho

Provider: Coastal Rehabilitation Services — Rivergate.

Course: 11/10/2025 through 03/10/2026 (17 weeks, 34 sessions).

Discharge status: Goals met. Full weight-bearing achieved. Right lower extremity strength within 90% of contralateral. Gait symmetrical. Patient tolerating full clinical-rotation activity. Home exercise program provided.

Signed: Melissa Ortiz, PT, DPT — 03/10/2026

Itemized Billing Summary — Devin Cho

Service line	Provider	Charges billed	Amount paid
Emergency department services 10/18/2025	Rivergate Regional Trauma Center	\$9,420	\$6,140
Diagnostic imaging (CT head, CT c-spine, XR tibia/fibula, CXR)	Rivergate Regional Trauma Center — Radiology	\$6,880	\$4,205
Inpatient room & board (4 days, med/surg)	Rivergate Regional Trauma Center	\$18,200	\$11,450
Operating room, anesthesia, PACU (10/19/2025)	Rivergate Regional Trauma Center	\$22,750	\$14,880
Orthopedic hardware (IM nail, plate/screws)	Rivergate Regional Trauma Center	\$11,900	\$7,620
Surgeon professional fee (ORIF)	Halloran Orthopedic Trauma Medical Group	\$14,500	\$9,240
Trauma surgery professional fees	Ramaswamy & Associates	\$6,300	\$4,100
Neurology consult / follow-up (2 visits)	Portner Neurology PC	\$1,875	\$1,120
Orthopedic follow-up (4 visits)	Halloran Orthopedic Trauma Medical Group	\$2,200	\$1,340
Physical therapy (34 sessions)	Coastal Rehabilitation Services	\$10,880	\$6,720
Pharmacy (inpatient and discharge)	Rivergate Regional Trauma Center	\$1,940	\$1,185
Total		\$106,845	\$68,000

Estimated total paid medical to date: \$68,000.

Estimated wage/stipend loss (nursing program clinical stipend and part-time employment during 8-week restriction and reduced-hour period): **\$9,200.**

SECTION B — ALICIA MOWBRAY**Mowbray, Alicia**MRN: 004182774 DOB: 01/30/1985 Sex: F
Adm: 10/18/2025 02:52 D/C: 10/18/2025 08:15 Coverage: On file**Patient Demographics**

Patient name	Alicia Mowbray
Date of birth	01/30/1985
Address	719 Cedar Street, Rivergate, CA 91766
Telephone	(626) 555-0155
Medical record number	004182774
Encounter type	Emergency department — treat and release
Arrival date/time	10/18/2025, 02:52 a.m.
Discharge date/time	10/18/2025, 08:15 a.m.
Attending of record	Julian K. Meade, MD — Emergency Medicine
Consulting	Karen L. Nagata, MD — Physical Medicine & Rehabilitation (bedside consult)

Presenting History (10/18/2025)

Patient is a 40-year-old home health aide who arrived by ground ambulance from the scene of the same collision, as a seated passenger on the ACTD Owl-service bus. Patient reports being seated on the right side of the bus, approximately mid-cabin, at the moment of impact. Describes being jolted forward against the seat back in front of her, then rebounding back into her own seat, with the head snapping forward and back. Denies loss of consciousness. Was able to walk off the bus with assistance to the sidewalk before EMS arrival.

Chief complaints: neck pain and stiffness; low-back pain; headache; pain and bruising over the left knee, left forearm, and right hip.

Past medical history: hypertension (well-controlled on lisinopril); no prior neck or back injuries; no prior fractures; no anticoagulants; no known drug allergies. Occupation involves patient transfers and lifting, typical loads 25–40 lb.

Emergency Department Assessment

Arrival vitals: BP 146/90, HR 92, RR 18, SpO₂ 99% on room air, Temp 36.7 °C. Alert, oriented ×3, GCS 15. In visible discomfort but not distress.

Focused examination:

- **Head/neck:** no scalp laceration. Marked paraspinal tenderness bilaterally over the cervical spine, worse at C5–C7 level. Reduced active range of motion in all planes secondary to pain. No midline bony tenderness. Negative Spurling maneuver. No neurologic deficit in upper extremities; strength 5/5 throughout; sensation intact; reflexes 2+ symmetric.
- **Back:** paraspinal tenderness lumbar region, worse at L3–L5 level. No midline bony tenderness. Straight-leg raise negative bilaterally. Strength and sensation intact in lower extremities.
- **Chest / abdomen:** no tenderness. Breath sounds clear. Abdomen soft.
- **Extremities:** 6 × 8 cm contusion over the left patella with mild effusion; range of motion of the left knee full, though painful at end-range flexion. Ligamentous testing (Lachman, anterior drawer, varus/valgus) negative.

Contusion left volar forearm, 4 × 5 cm. Contusion right greater trochanter, 5 × 6 cm. No open wounds. Distal neurovascular status intact throughout.

Imaging (10/18/2025)

- **Radiograph, cervical spine (AP, lateral, open-mouth odontoid):** no fracture, no malalignment. Preservation of normal lordosis mildly diminished, consistent with muscular spasm.
- **Radiograph, lumbar spine (AP, lateral):** no fracture, no acute abnormality.
- **Radiograph, left knee (AP, lateral, sunrise):** no fracture; small suprapatellar effusion.

Advanced imaging deferred; no red-flag features on examination or plain films.

Emergency Department Diagnoses

1. Cervical strain (whiplash-associated disorder, Grade II) (S13.4XXA)
2. Lumbar strain (S39.012A)
3. Contusion, left knee, with small effusion (S80.02XA)
4. Contusion, left forearm (S50.11XA)
5. Contusion, right hip / greater trochanter (S70.02XA)
6. Headache, post-traumatic tension-type (G44.309)

Emergency Department Course and Disposition

Patient was treated with intramuscular ketorolac and oral cyclobenzaprine in the department with symptomatic improvement. Ambulation trial was successful with mild antalgic gait favoring the left lower extremity. Neurovascular exam remained intact throughout observation. PM&R consulted for early conservative-care planning; recommended activity as tolerated with soft cervical collar for comfort during transport, referral to outpatient physical therapy, and short-course NSAID/muscle-relaxant regimen. No indication for admission.

Disposition: Discharged home in the care of a family member on 10/18/2025 at 08:15 a.m.

Medications on discharge:

- Ibuprofen 600 mg PO q8h with food × 10 days, then PRN
- Cyclobenzaprine 5 mg PO TID PRN muscle spasm × 7 days
- Acetaminophen 650 mg PO q6h PRN pain

Activity and restrictions:

- Soft cervical collar for comfort only, not to exceed 72 hours, discontinue for sleep.
- Ice to the left knee 20 minutes q2h while awake for the first 48 hours; heat to neck and back after 48 hours as tolerated.
- **Work restriction: no work for one (1) week; light duty (no lifting greater than 10 lb, no patient transfers, no repetitive bending or overhead reaching) for an additional two (2) weeks** — total three (3) weeks light-duty restriction from date of injury pending re-evaluation.
- Driving restricted while taking cyclobenzaprine; resume when off sedating medication and able to turn head fully.
- Home-based active range-of-motion exercises for neck and back as tolerated; instruction sheet provided.

Follow-up arranged:

- Primary care follow-up within 7–10 days.
- Outpatient physical therapy referral (Coastal Rehabilitation Services) to begin within 7 days.
- Return to ED for worsening pain, new neurologic symptoms, numbness/tingling in the extremities, bowel or bladder dysfunction, or fever.

Electronically signed: Julian K. Meade, MD — Emergency Medicine, 10/18/2025, 08:22 a.m.

Co-signed (PM&R consult): Karen L. Nagata, MD, 10/18/2025, 07:58 a.m.

Follow-Up and Physical Therapy — Alicia Mowbray

Primary care follow-up — 10/24/2025 (Dr. Sonia Bhatt, Rivergate Family Medicine). Patient reports moderate improvement with medications and rest. Neck stiffness persists; low-back pain improving. Contusions fading. Cleared to begin outpatient physical therapy. Light-duty work restriction continued as previously written. Return in three weeks or sooner as needed.

Physical therapy course. Coastal Rehabilitation Services — Rivergate. Initial evaluation 10/27/2025. Twice-weekly sessions × 6 weeks, total 12 sessions, through 12/05/2025. Treatment: cervical and lumbar range-of-motion, manual therapy, postural retraining, progressive strengthening, work-conditioning simulation (patient-transfer mechanics).

Physical therapy discharge — 12/05/2025 (Nathan Choi, PT, DPT). Cervical range of motion within 5° of pre-injury baseline; lumbar range of motion full. Pain reported as 1–2/10 on activity, 0/10 at rest. Patient demonstrating safe body mechanics for occupational lifting and transfers. Discharged with home exercise program.

Primary care re-evaluation — 11/07/2025 (Dr. Bhatt). Full-duty release granted effective 11/08/2025 (three weeks post-injury). No permanent restrictions.

Signed: Sonia Bhatt, MD — Rivergate Family Medicine, 11/07/2025

Itemized Billing Summary — Alicia Mowbray

Service line	Provider	Charges billed	Amount paid
Emergency department services 10/18/2025	Rivergate Regional Trauma Center	\$4,180	\$2,720
Diagnostic imaging (XR c-spine, XR l-spine, XR knee)	Rivergate Regional Trauma Center — Radiology	\$2,340	\$1,510
Emergency medicine professional fee	Meade Emergency Physicians Group	\$1,120	\$720
PM&R; bedside consult	Nagata Physical Medicine Associates	\$640	\$410
Primary care follow-up (2 visits)	Rivergate Family Medicine	\$520	\$335
Physical therapy (12 sessions)	Coastal Rehabilitation Services	\$9,240	\$6,540
Pharmacy (discharge medications)	Rivergate Regional Trauma Center	\$410	\$265
Total		\$18,450	\$12,500

Estimated total paid medical to date: \$12,500.

Estimated wage loss (one week off work plus reduced hours over two weeks of light duty at usual home-health-aide schedule): **\$1,800.**

Records Custodian Certification

I, Renata Wills, RHIA, Director of Health Information Management for Rivergate Regional Trauma Center, hereby certify that the foregoing records are true and correct copies of the medical records of Devin Cho (MRN 004182773) and Alicia Mowbray (MRN 004182774) as maintained by this facility in the ordinary course of business, and that the entries were made at or near the time of the acts, conditions, or events described by persons with knowledge of those matters.

Signed: Renata Wills, RHIA — Director, Health Information Management

Rivergate Regional Trauma Center

2100 Medical Center Drive, Rivergate, CA 91766

Date: 03/12/2026